

Pembrolizumab + pemetrexed + cisplatin

Lung - NCCP 00569 - version 5

Your medicines

Pembrolizumab - Pemetrexed - Cisplatin

Your treatment schedule

Pembrolizumab 200 mg + pemetrexed 500 mg/m² + cisplatin 75 mg/m² every 21 days.

A picture of side effects

Tell your team about new or worsening symptoms. Numbers link each symptom box to the body map.

Pembrolizumab - immune effects

Pemetrexed / cisplatin - chemotherapy

1 Hormones / brain

Pembrolizumab

Unusual fatigue, dizziness, headache, thirst or passing more urine can indicate immune endocrine toxicity.

3 Heart

Pembrolizumab

Chest pain, palpitations, fainting or new breathlessness needs urgent assessment.

5 Kidneys / hydration

Pemetrexed / cisplatin / pembrolizumab

Kidney function is monitored closely. Cisplatin needs hydration; immune nephritis can also occur.

7 Blood / infection

Pemetrexed / cisplatin

Fever, shivers, bruising or sudden illness can reflect low blood counts and needs prompt contact.

2 Lungs

Pembrolizumab

New cough or breathlessness needs prompt assessment for pneumonitis and other causes.

4 Liver / bowel

Pembrolizumab

Yellow eyes, dark urine, persistent diarrhoea or abdominal pain can signal immune inflammation.

6 Hearing / nerves

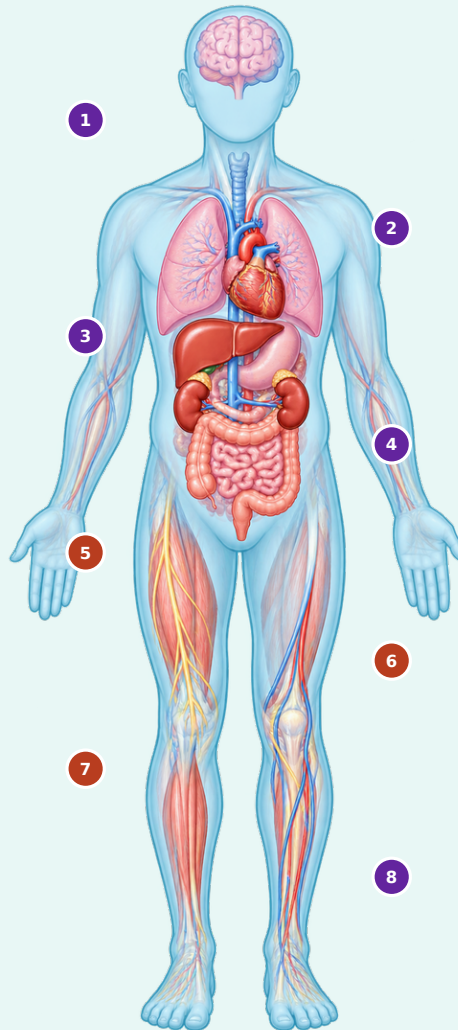
Cisplatin

Report tinnitus, hearing change, numbness or tingling before it worsens.

8 Skin / mouth

Pembrolizumab / pemetrexed

Report severe or blistering rash and mouth soreness affecting fluids.



Selected effects only. The map does not show likelihood and cannot diagnose the cause of a symptom.

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Your treatment journey

Cycles 1-4 - day 1: Pembrolizumab 200 mg + pemetrexed 500 mg/m² + cisplatin 75 mg/m² every 21 days.

Cisplatin phase: Cisplatin is given in cycles 1-4 with hydration and electrolyte support.

Maintenance: After the platinum phase, pembrolizumab + pemetrexed continue every 21 days when appropriate.

Throughout: Immune toxicities can occur during treatment or after treatment stops.

Monitoring and support

- Folic acid and vitamin B12 support are required with pemetrexed.
- Cisplatin requires hydration/electrolyte management and high-emetogenic-risk antiemetic support.
- Blood count, kidney/liver profile and glucose are checked; thyroid monitoring continues during pembrolizumab.
- Report immune symptoms early rather than waiting for the next cycle.

WHEN TO CALL

Contact oncology urgently for fever, new breathlessness, persistent diarrhoea, jaundice, severe rash, chest pain, marked weakness, inability to maintain fluids, reduced urine or new neurological symptoms. For severe breathing difficulty, collapse, seizure or severe chest pain call 112/999.

Consent discussion support

Use the approved consent process to document treatment intent, expected benefit, material risks, alternatives, questions and the patient's decision.

Scan to open the regimen-specific patient portal

<https://sactcheck.com/patient/00569/>

Official NCCP 00569 v5

SACTCheck v0.80.4 - patient information prototype for clinical review. Follow your oncology team and local emergency instructions.

